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|----------|-----------------------------------------------------------------------------------|-----------------------------------------------------------------------------------|----------|
| 품명       | 글리클리짓정-설명서                                                                        |                                                                                   |          |
| 작업자      | 총무아트                                                                              | 010-2111-2984                                                                     |          |
| 규격       | (전)                                                                               | (후)                                                                               | 90×210mm |
| 수출<br>국가 | 싱가포르                                                                              | 완료<br>일자                                                                          |          |
| 인쇄       |  |  | 별색 2도    |

## Sam Chun Dang GLYCLAZIDE TABLET

(Glyclazide 80mg)

### ■ Composition

Each tablet contains  
Glyclazide ----- 80mg

### ■ Description

Glyclazide Tablet is a white, round tablet with a logo 'G' on one side and 'SCD' on the other side, which comes in a transparent blister of 10 tablets.

### ■ Indication

Treatment of non-insulin-dependent diabetes mellitus.  
(only when diet and exercise fail to control hyperglycemia).

### ■ Route of Administration

Oral route

### ■ Dosage and Administration

The usual initial dose is 40 to 80mg daily, gradually increased, if necessary up to 320mg daily.  
Dose of more than 160mg daily should be given in 2 divided dosage.

The dose may need to be titrated if patient has been switched from a different brand of product containing Glyclazide

### \*Pharmacology

The sulfonylurea cause hypoglycemia by stimulating release of insulin from pancreatic  $\beta$  cell and by increasing the sensitivity of peripheral tissues to insulin. The predominant effect is on insulin secretion.

Sulfonylureas stimulate release of insulin from the isolated perfused pancreas, isolated islets, and insulinoma cells in culture. The drugs are ineffective in pancreatectomized animals or patients and in diabetic patients who have no endogenous insulin. The effect of the sulfonylureas appear to be initiated by drug interaction with cell-surface receptors on the pancreatic  $\beta$  cell. This result is reduced conductance of ad ATP-sensitive  $K^+$  channel.

The drugs thus resemble physiological secretagogues, which also lower the conductance of this channel. Reduced  $K^+$  conductance causes membrane depolarization and influx of  $Ca^{2+}$  through voltage-sensitive  $Ca^{2+}$  channels. It appears likely that the receptor for the sulfonylureas is the ATP-sensitive  $K^+$  channel itself.

### \* Pharmacokinetics

Glyclazide is readily absorbed from the gastro-intestinal tract.

It is extensively bound to plasma proteins.

The half-life is about 10 to 12 hours.

Glyclazide is extensively metabolised in the liver to metabolites without significant hypoglycaemic activity.

Both unchanged drug and metabolites are excreted in the urine.

### ■ Precautions and Adverse Reaction

#### 1. Warnings

Because this drug can cause severe or delayed hypoglycemia, careful management is recommended.

#### 2. Contraindications

- 1)Patients with severe ketosis, diabetic coma, insulin-dependent diabetes mellitus
- 2)Patients with severe hepatic, or renal failure
- 3)Patients with severe infections, before and after operation, severe traumatic patients.
- 4)Patients with gastrointestinal problems such as diarrhea, or vomiting.
- 5)Patients with hypersensitivity to this drug or sulfonylureas.

#### 3. Adverse Reactions

- 1)Hypoglycemia : Unusual tiredness, excessive hunger, sweating, palpitation, tremor, headache, paraesthesia, anxiety, excitation, nervousness, difficulty in concentrating, mental disturbance, conscious disturbance, or convulsion may occur.
- 2)Blood : Anemia or leukopenia may infrequently occur.
- 3)Liver : Hepatic dysfunction may infrequently occur.
- 4)Kidney : Elevation of BUN or serum creatinine may infrequently occur.
- 5)Stomach : Anorexia, nausea, or vomiting may infrequently occur, gastric flatus, abdominal pain, diarrhea, or constipation may rarely occur.
- 6)Anaphylaxis : Itching, or rash may infrequently occur, photosensitivity may rarely occur.
- 7)Others : Head fullness, headache, dizziness, or heat may rarely occur.

#### 4. General Precautions

1)This drug should be used in patients who have been diagnosed as diabetic. In case with symptoms like diabetes such as abnormal glucose tolerance, positive urine glucose, careful administration is needed.

2)This drug should be used only when diet and exercise therapy fail to control hyperglycemia.  
3)Begin with small dosage when taking this drug. Check blood-glucose and urine-glucose regularly to see the effect of this medicine and turn to other therapy in case of insufficient effect.

4)Constantly check the quantity of meals, weight change, blood-glucose level infection to judge whether you should continue taking this medicine and its dosage.

5)Patients who work on high places or drive cars should taking the medicine with caution since it can cause severe hypoglycemia. Be cautious of hypoglycemia and the patient and his family as well must pay attention.

#### 5. Drug Interactions

Descending of blood glucose can increase or decrease when the following medicines are taken at the same time. In this case, observe the patient and his blood glucose level carefully.

##### 1)Increasing medicines

Insulines, biguanides, pyrazolons(phenylbutazone etc.) probenecid, coumarins, salicylates(aspirin etc.),  $\beta$ -blockers(propranolol), MAO inhibitors, sulfonamides, dihydroergotamines, chloramphenicol, tetracycline's antibiotics, clofibrate.

##### 2)Decreasing medicines

Epinephrine, Adrenocorticoids, thyroid hormones, estrogens, diuretics, pyrazinamide, isoniazide, nicotinic acid, phenothiazines.

#### 6. Use during pregnancy

Safety during pregnancy has not been established, this drug should not be administered to patients who are or may become pregnant.

Also, hypoglycemia has been reported in neonates born to mothers who were receiving a sulfonylurea.

### \*Overdosage: Syptoms & its treatment

Overdose may result in hypoglycemia

Symptoms are : Anxiety, chills, cold sweats, confusion, cool pale skin, difficulty in concentration, drowsiness, excessive hunger, fast heart beat

In acute poisoning, the stomach should be emptied by aspiration and lavage, The patient should be observed over several days in case hypoglycemia recurs. If symptoms of mild hypoglycemia occur, patient should eat or drink a source of sugar and obtain medical assistance immediately. If glycemic coma is diagnosed, patient should be given intravenous glucose or, if this not practicable, subcutaneous or intramuscular glucagon.

### ■ Packing

6 blisters x 10 tablets in a box with leaflet

### ■ Storage

Store below 30°C.

### ■ Shelf life

3 years from manufacturing date.

### ■ Revised Date

Oct. 2018



Manufactured by  
**SAMCHUNDANG PHARM.CO.,LTD.**  
Seoul, Korea

Marketing Authorization holder  
**The Zyfas Medical.Co.**  
NO.7, Jalan Molek 3/10 Taman Molek 81100  
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